

DCD / WLST COMFORT MEDICATION — BEDSIDE CARD

[HOSPITAL] SICU · DRAFT v1 · 2026-07-08 — pending P&T/Ethics approval
Adults · defaults are for the opioid/benzodiazepine-naïve · identical for donors & non-donors

1 · BEFORE WITHDRAWAL — all boxes checked before family enters

- ☐ **Continue all current sedation/analgesia infusions.** Never weaned for donation.
- ☐ **No paralytics — ever, from the WLST decision on.** Prior NMB: TOF $\geq$ 3/4 and motor recovery, documented.
- ☐ Functioning **arterial line** + 2 reliable IV routes.
- ☐ **3 hours of medication at the bedside:** morphine $\geq$ 60 mg · midazolam $\geq$ 20 mg · glycopyrrolate · propofol vial · flushes.
- ☐ Baseline **RDOS / CPOT / RASS** charted.
- ☐ Time-out done: roles · no recovery staff present · stand-down plan · **ICU bed held.**

2 · PREMEDICATE — anticipatory dosing is endorsed practice; chart it

When	Give
T-30 min	Glycopyrrolate 0.4 mg IV $\times$ 1 — chart “secretion control for comfort” (expect mild HR rise; side effect, not a target)
T-10-30 min	Morphine 5 mg IV (2–10 mg $\approx$ 0.05–0.1 mg/kg) + Midazolam 2 mg IV (1–5 mg)
Tolerant pt	Bolus = 1 hour of the current infusion (or 10–15% of 24-h opioid total) — chart the rationale
T-0	Withdraw only when RDOS < 3 . If $\geq$ 3 $\rightarrow$ treat first (Box 4), then proceed.

Heparin (donation med — separate authorization, care-team RN gives): **300 units/kg IV (~30,000 U)** at extubation per DNAZ agreement. **Hold if actively bleeding** $\rightarrow$ attending.

3 · RDOS — score all 8 · total 0–16

Item	0	1	2
Heart rate	<90	90–109	$\geq$ 110
Resp rate	$\leq$ 18	19–30	>30
Restlessness	none	occasional	frequent
Paradoxical breathing	none	—	present
Accessory muscles	none	slight	pronounced
End-exp grunting	none	—	present
Nasal flaring	none	—	present
Fearful face	none	—	present

0–2 none · **3** mild — **TREAT** · **4–6** moderate · **$\geq$ 7** severe — escalate urgently. **Titrate to RDOS < 3.**

4 · TREAT WHEN — any ONE trigger · score + chart q10 min and 10 min after every dose

RDOS $\geq$ 3 · **CPOT > 2** (grimace, tension, guarding) · **RASS $\geq$ +1** / restlessness · **HR $\uparrow$ >20% with any distress sign** · new diaphoresis / labored breathing

5 · TITRATION LADDER

Morphine 2–5 mg IV q10 min PRN	Ineffective at 10-min recheck $\rightarrow$ escalate 2 $\rightarrow$ 4 $\rightarrow$ 6 $\rightarrow$ 8 mg (keep doubling pattern while triggered)
Midazolam 1–2 mg IV q10 min PRN	agitation/anxiety signs $\rightarrow$ escalate to 4 mg
$\geq$ 3 opioid boluses in 1 h	start infusion at $\approx$50% of the hour’s effective bolus total (mg/h) — keep bolusing with every rate increase; a rate change alone is too slow
Already on infusions	bolus = 1-h infusion equivalent + increase rate 25–50% $\rightarrow$ recheck 10 min
Glycopyrrolate 0.2 mg IV q2h PRN	secretions / death rattle; reposition; suction only if it doesn’t distress
Refractory distress	attending to bedside $\rightarrow$ propofol 20–50 mg IV $\rightarrow$ 10–100 mg/h (or up-titrate existing propofol/dexmedetomidine)

Endpoint = comfort: RDOS < 3 · CPOT $\leq$ 2 · settled facies/body. **No ceiling dose** — and no dose without a charted trigger. The endpoint is never a target RR, BP, or time.

NEVER

- ☒ Paralytics at/after the WLST decision — none, ever
- ☒ Doses timed to the donation clock, OR schedule, or organ viability
- ☒ Dosing to an isolated vital sign in a comfortable-appearing patient — hypotension, bradycardia, agonal pattern without distress = dying, not suffering
- ☒ OPO / recovery / transplant staff ordering, giving, or advising comfort care
- ☒ Weaning sedation to “protect” the timeline
- ☒ KCl, Mg boluses, or anything with no comfort indication

CHART IT LIKE THIS — every dose, no exceptions

1310 – RDOS 6 (RR 32, accessory use, restless, HR 112).
Morphine 4 mg IV for respiratory distress.
1320 – RDOS 2. Appears comfortable.

Time · objective finding (score + components) · drug/dose/route · **comfort indication** · 10-min reassessment score. RN flowsheet and MD note must agree.

6 · DETERMINATION OF DEATH

Arrest

Art-line pulse pressure = **0**
+ apnea + unresponsive (palpation never suffices)



5:00 hands-off

Continuous art line + ECG. Pulsatility returns $\rightarrow$ **restart clock**
· ECG complexes *without* pulse pressure do NOT interrupt.



Pronounce

Physician not on OPO/recovery/transplant team. TOD = end of observation. Death note per policy §9.4.



Then

Recovery team may enter — never before. Family escorted out at arrest, before team entry.

No death by [120 min / DNAZ organ window]: stand down $\rightarrow$ return to held ICU bed $\rightarrow$ **this same protocol continues unchanged.** Tissue donation may remain possible. **Anyone may call a pause** for a patient concern at any point.

ONE PROTOCOL FOR EVERY WITHDRAWAL — DONOR OR NOT. Symptom-triggered, documented, no ceiling, no clock. That is the legal shield.

Sources: ACCM/Truog Crit Care Med 2008 · ATS/ISHLT/SCCM/AOPO/UNOS 2013 · VHA Directive 1102.07 · Fast Fact #34 (4th ed 2025) · Campbell RDOS (cut-points Palliat Med 2015) · Dhanani NEJM 2021 (longest autoresuscitation 4:20 $\rightarrow$ 5-min observation) · SILENCE RCT JAMA 2021 · Vacco v. Quill, 521 U.S. 793 · A.R.S. §14-1107. Full policy + citations: **DRAFT_DCD_policy_comfort_med_protocol.md**. Not for use until approved.